

--SUMMARY--

Decision No. 2342/15

18-Nov-2015

J.Dimovski

- Subsequent incidents (outside work)

The worker suffered a knee injury in October 2009, when he slipped. In January 2011, the worker fell on snow outside his home, suffering further injury. The worker appealed a decision of the Appeals Resolution Officer denying entitlement for the fall outside his home in January 2011.

The worker submitted that the fall was a secondary condition resulting when his knee gave out. However, the Vice-Chair found that medical evidence did not support the claim. Evidence indicated that the ligaments in the worker's knee were stable. The accident at home in January 2011 was a new, non-compensable accident. The appeal was dismissed.

6 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w0116s]

Style of Cause:

Neutral Citation: 2015 ONWSIAT 2632



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 2342/15

BEFORE:

J. Dimovski: Vice-Chair

HEARING:

October 30, 2015 at Toronto
Oral

DATE OF DECISION:

November 18, 2015

NEUTRAL CITATION:

2015 ONWSIAT 2632

DECISION(S) UNDER APPEAL: WSIB ARO decision dated January 29, 2013

APPEARANCES:

For the worker:

Mr. A. Barclay, Office of the Worker Adviser

For the employer:

E.A., Employer

Interpreter:

N/A

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

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REASONS**(i) Introduction to appeal proceedings**

[1] On October 9, 2009, this now 56 year old worker sustained a left knee injury after he slipped disembarking from his truck while in the course of his employment as a truck driver. He went off work and he received physiotherapy treatment. After an unsuccessful attempt to return to work and a period off work, he returned to a modified office job before returning to truck driving duties in October 2010. As a result of his ongoing complaints, he underwent arthroscopic surgical and debridement procedure on June 13, 2011. The Board determined that the recommended surgery was a work-related procedure and thus compensable.

[2] On January 15, 2011, prior to his surgery, he fell outside his home injuring his left knee. The worker claimed that he fell because his left knee gave out. He argued that it gave out because of his functional and physical impairment caused by his 2009 accident. As a result, the worker claimed his 2009 compensable injury significantly contributed to his 2011 home accident and as such his injuries arising from his 2011 home accident should be accepted as compensable secondary conditions. As he missed time from work as a result of his home accident, he also claimed loss of earnings (LOE) benefits from January 16, 2011 to June 13, 2011. The Board denied the worker's 2011 home accident was compensable. Since the Board accepted that the worker had performed his regular duties prior to his home accident, it also denied him LOE benefits from January 15, 2011 to June 13, 2011.

[3] As noted above, the worker underwent a left knee debridement on June 13, 2011. Although initially suspected as the cause of his symptoms and which prompted the need for his surgery, Dr. Casses' confirmed post-surgery that the worker's meniscus was intact and had not been torn. The worker's post-operative diagnosis was: "mild degree of chondromalacia involving the inferior pole of the patella and medial femoral condyle". Although the worker received several weeks of post-operative physiotherapy and LOE benefits, the Board determined any ongoing left knee impairment was non-work related and that his knee strain had resolved. The Board informed the worker that his June 2011 surgery was allowed as compensable to address whether he had a meniscus tear. As he did not have a tear and his surgery confirmed that he had pre-existing "chondromalacia, which is a degenerative condition of the knee joint", the Board denied him further benefits for the left knee since his pre-existing condition was likely unrelated to the twisting injury which "caused the original MCL strain on October 9, 2009". As a result, it found that the worker's MCL strain had resolved with a maximum medical recovery (MMR) date of June 11, 2011. Any ongoing impairment was the result of his non-compensable degenerative condition.

[4] The worker sought entitlement to LOE benefits between January and June 2011 and a NEL award for his injuries arising from his 2009 accident at the Board's final level of an appeal. In a decision dated January 29, 2013, an Appeals Resolution Officer (ARO) upheld the Board's decisions on these issues.

[5] Thus, the issues in dispute are as follows:

- whether the worker has LOE benefits from January 16, 2011 to June 13, 2011; and
- whether the worker has entitlement to a NEL award for his left knee arising from his compensable 2009 accident.

[6] At the hearing, the worker confirmed that the LOE benefit issue is related to whether the worker's impairment during the period is related to secondary injury/condition caused by his compensable 2009 accident. Specifically, the worker argues that his 2009 left knee injury persisted and significantly contributed to his fall at home in January 2011.

[7] Also, the worker was permitted to testify via teleconference.

(ii) Analysis

[8] Since the worker's initial compensable accident occurred in October 2009, the *Workplace Safety and Insurance Act, 1997* (WSIA), as amended, applies to this appeal. Section 46 of the WSIA provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[9] "Impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss. "Permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery.

[10] Pursuant to section 126 of the WSIA, I have considered the policies forwarded by the Board as necessary in deciding the issues in this appeal. *Operational Policy Manual* (OPM) Document No. 11-01-05, entitled "Determining Maximum Recovery (MMR)" provides that workers reach MMR when they have reached a plateau in their recovery and it is not likely that there will be any further significant improvement in their medical impairment.

[11] Until his June 2011 arthroscopy ruled it out, as set-out by Dr. Casses' (orthopaedic) report dated September 14, 2010, the worker's physicians suspected that his presentation was the result of a combination of osteoarthritic degeneration and possible meniscus tear. However, in spite of sustaining additional left knee trauma in January 2011, Dr. Casses denied any ligament injury, especially with his meniscus, in his post-operative report dated June 13, 2011. Since the Board accepted that the reason for the arthroscopy was to confirm whether he had damaged his meniscus, he was provided limited LOE benefits as recognition that his recuperation from surgery was in part due to a work-related reason (to determine if he had a meniscus tear).

[12] The Board determined that his ongoing presentation was due to chondromalacia, a non-compensable pre-existing degenerative condition. The employer submitted that the Board's reasoning should be upheld. I acknowledge that there is no medical opinion on record which supports the development of the worker's underlying osteoarthritic condition was work-related. In fact, Dr. Smyth denied that the worker's underlying condition developed as a result of his accident. Mr. Barclay, however, relied on Dr. Smyth's reports to argue that the worker's compensable accident likely aggravated his underlying condition. Although the worker had pre-disposing factors (i.e. obesity) which could have initiated his symptoms, the Board accepted that the worker did not have any left knee symptoms prior to his accident. It also accepted that the worker's accident was sufficiently traumatic enough to have caused persistent and unabated left knee symptoms which required arthroscopic investigation. There is no evidence to support the worker's non-compensable condition was triggered in isolation to the trauma of his compensable accident. Further, I do not find any medical opinion on record which allows this Tribunal to delineate or divide the worker's presentation into two different or distinct conditions developed from separate initiating events, i.e. a compensable strain caused by a compensable accident and the onset symptoms due to the natural progression of his non-compensable pre-existing

condition. As such, I find that it was likely that the worker's underlying pre-existing condition (whether with an additional knee strain component or not) was aggravated by the worker's compensable accident.

[13] I also find that the preponderance of evidence on record from prior to his January 2011 home accident supports his compensable accident likely resulted in permanent and functional impairment. The worker's representative noted that the worker's inability to return to the driving duties that required him to drive a manual truck supports his condition reflected a permanent impairment. If the worker could not return to his pre-accident duties, this would be a factor supporting his permanent impairment. On this point, however, I found the worker's testimony uncertain. He could not confirm the details of his job with certainty in spite of several attempts to ascertain his job duties prior to his 2011 home accident. He was a poor historian. As a result, aside from the confusion surrounding the nature of his duties, I relied on the documentary evidence on record which supports he had functional and physical impairment prior to January 2011. In this regard, I rely on Dr. Casses' medical report dated September 14, 2010, which noted that the worker experienced very good range of motion on flexion and extension with a "mild degree of discomfort" on rotational movements of the left knee. This report supports the worker had an impairment. Given Dr. Casses' personal examination of the worker came after extensive physiotherapy treatment and almost two years after his accident, I also find that the worker's impairment is now permanent. Therefore, I find that the worker has entitlement to a NEL award for his left knee.

[14] I now turn to whether the worker's January 2011 home accident is compensable. Section 43 (1) of the WSIA provides that a worker who has a loss of earnings as a result of a work-related injury has entitlement to LOE benefits. With regard to the worker's LOE claim, I have paid particular attention to *Operational Policy Manual* (OPM) Document No. 15-05-01. This policy provides that workers sustaining secondary conditions that are casually linked to a work-related injury will "derive benefits to compensate for the further aggravation of the work-related impairment or for new injuries". A compensable knee giving-out causing a fall and causing further injury would land within the purview of this policy. OPM Document No. 15-05-03, entitled "Non-Work-Related – Second Accidents" notes that "[i]f an injured worker has a second accident while receiving WSIB benefits, but the second accident is **not caused** by a work-related injury, there is no entitlement for the second accident".

[15] The worker argues that his left knee impairment caused or significantly contributed to his fall at home in 2011. In his testimony, the worker stated that while outside his home in January 2011 his left knee gave-out or "collapsed" and he fell to the snow covered (over ice) ground. The more contemporaneous reporting noted that the worker had "slipped" on ice. This included the emergency reporting from January 16, 2011. Slipping on ice, at home, is not a work-related injury unless it was caused by some work-related reason. The worker stated that his left knee gave-out because his compensable injury caused or significantly contributed to his home accident. Although I acknowledge that the worker informed Dr. Casses' that his left knee felt like it could give out, and thus consistent with an injuring mechanism that likely caused or contributed to his knee "collapsing", I find there is no objective medical evidence or opinion which supports the worker's left knee ligaments were sufficiently weakened or unstable to have caused his knee to have collapsed. On the contrary, the May 2010 MRI report indicated the worker's ligaments were intact. Even after his home accident, Dr. Casses' did not find any ligament instability in the course of his arthroscopic procedure. In his report dated July 7, 2011,

Dr. Casses, in fact, noted “[l]igamentous wise, the knee was completely stable”. Given the state of the worker’s ligaments even after his 2011 trauma, I do not accept that the nature of the worker’s knee condition made him susceptible to knee collapses. As there is also no persuasive medical opinion on record to support this mechanism of injury, I placed little weight on the worker’s testimony. Instead, I preferred that the contemporaneous emergency reports on record likely recorded the reasons for the worker’s fall in January 2011 since they were based on the information supplied by the worker closer to the event in question. As such, I find that the worker’s fall at home in January 2011, was a new accident that was not related to the worker’s compensable 2009 left knee injury. Since the worker had returned to his pre-accident job (with or without some accommodation), his absence from work, and thus his loss of earnings from January to June 2011, was likely due to his non-compensable home accident. Therefore, the worker is not entitled to LOE benefits from January 15, 2011 to June 13, 2011.

[16] For further clarity, the worker’s 2011 home accident was a non-compensable intervening event. The Board is to determine the worker’s NEL award in light of the fact that any permanent impairment flowing from his 2011 home accident is non-compensable. The quantum of the worker’s NEL award arising from his 2009 compensable accident is referred back to the Board for further adjudication.

[17] I also note that in light of my findings set-out above, the employer might have entitlement to claim cost relief. The employer may pursue any relief with the Board.

DISPOSITION

[18] The appeal is allowed in part.

[19] The worker has entitlement for a left knee NEL award. The quantum of the worker's NEL award arising from his 2009 compensable accident is referred back to the Board for further adjudication.

[20] The worker's January 15, 2011 accident is a non-compensable intervening event. The worker does not have entitlement to LOE benefits from January 15, 2011 to June 13, 2011.

DATED: November 18, 2015

SIGNED: J. Dimovski